Medbury Healthcare Group

The revenue architecture

Where the money comes from, and what we keep.

Four engines, two businesses, one building. Stabilised figures, which the campus reaches in year three.

Medbury Healthcare Group

Four engines, two businesses.

Rent
68
Three tenants

Sessional
370
Rooms and membership

Theatre
152
Own and external lists

Own clinics
1,008
Four companies

THE CAMPUS, a platform business
590 M revenue / 186 M EBITDA
Rent, rooms and theatre. Medbury owns all of it

THE CLINICS, operating
1,008 M revenue / 274 M EBITDA
Four companies, two of them shared

Group 1,598 M revenue, 460 M EBITDA, 29% margin

All figures NGN M a year, stabilised

What each one is, and how it behaves.

Engine	NGN M	What it is	How it behaves
Rent	68	Three tenants: the conversion clinic, diagnostics and the pharmacy	Contracted. The floor under everything else
Sessional	370	Room fees and membership from visiting consultants	Recurring, and scales with the panel rather than with capital
Theatre	152	Own lists, plus external surgeons paying by the list	The highest value hour in the building, and the fastest to pay back
Own clinics	1,008	Aesthetics, hair restoration, bariatric and metabolic, longevity and infusion	Clinical revenue. Carries the staffing and the clinical risk

The first three need no clinicians on our payroll. They are paid for space, time and equipment. The fourth is a different kind of business sitting in the same building.

They look alike and they are not.

	The campus	The clinics
Revenue	590 M	1,008 M
EBITDA	186 M	274 M
Margin	31.5%	27.2%
What it sells	Space, time and equipment	Clinical care
Who it employs	Reception, facilities, a booking team	Doctors, nurses, clinical staff
What can go wrong	Rooms sit empty	A clinical outcome, a key clinician leaving, a regulator
Medbury's share	All of it	All of two, part of the other two
Does it travel	Yes. It is the template for the second site	Each one has to be rebuilt

The platform earns a higher margin than the clinical operations, on less risk and with no clinicians on the payroll. That is unusual and it is worth protecting.

After the minority stakes, the picture changes.

The campus	185.7	100%	185.7
Medlyfe Aesthetics and Regenerative	82.0	51%	41.8
Lyfe Place Hair Restoration	55.0	100%	55.0
Bariatric, Metabolic and Endoscopic	66.0	70%	46.2
Medlyfe Longevity and Infusion	71.0	100%	71.0
Attributable to Medbury			**399.7**

The campus is 37% of group revenue but 46% of what Medbury keeps. It earns a better margin and we own every naira of it, while two of the four clinics are shared. The property business is quietly the better business.

Medbury Healthcare Group

The two halves fail in different ways.

	What would hurt it	What protects it
Rent	A tenant leaving or paying late	Five year terms, six month deposits, and two of the three tenants are our own businesses
Sessional	The panel not filling. This is the single largest assumption in the plan	Price to fill rather than to maximise. Founding rates held for three years
Theatre	Lists not booked, or the surgical anchor not signing	Hold the capital until an anchor is signed, then it pays back inside a year
Own clinics	Clinical outcome, a key clinician leaving, regulatory action	Each in its own company, so a problem in one does not reach the others

Putting each clinic in its own company is not tidiness. It is what stops a clinical problem in one business reaching the building, the tenants and the other three.

Medbury Healthcare Group

What follows from the shape

- **Protect the platform margin**
  Related party rents set at arm's length, and the clinics not allowed to crowd sessional capacity out of the rooms they do not pay for.

- **Grow the platform before the clinics**
  A naira of platform EBITDA is worth more to Medbury than a naira in a clinic we share, and it carries less risk to earn.

- **Gate the theatre on a signed anchor**
  It is the fastest payback in the building and the easiest to strand.

- **Build the first site to be copied**
  The rate card, the service model and the protocols are the asset. The second site inherits them rather than inventing them.